PREOPERATIVE DIAGNOSIS: Chronic medically refractory rhinosinusitis with bilateral massive nasal polyposis (CRSwNP) and pan-sinusitis.

POSTOPERATIVE DIAGNOSIS: Chronic hyperplastic rhinosinusitis with diffuse nasal polyposis.

PROCEDURE: Bilateral functional endoscopic sinus surgery (FESS), including bilateral total ethmoidectomy, bilateral maxillary antrostomy, bilateral sphenoidotomy, and frontal sinusotomy (Draf IIa).

SURGICAL PROCEDURE: Following induction of general endotracheal anesthesia, the patient was positioned supine with head elevated 15 degrees. Bilateral nasal cavities were decongested with pledgets soaked in 4% cocaine and 1:1,000 epinephrine for 10 minutes. Under visual control utilizing a 0-degree Karl Storz rigid nasal endoscope, 1% lidocaine with 1:100,000 epinephrine was injected into the uncinate process, middle turbinate root, and sphenopalatine regions bilaterally.

Right Nasal Cavity: Extensive polypoid disease completely obstructing the middle meatus was identified. Using a microdebrider (Medtronic Straightshot with 4 mm Tricut blade at 3,000 rpm), polyps were debulked. The uncinate process was identified and resected using a sickle knife and back-biting forceps. The natural ostium of the right maxillary sinus was identified and enlarged posteriorly and inferiorly into a wide antrostomy; thick, mucopurulent secretions were evacuated. An anterior and posterior ethmoidectomy was performed, meticulously skeletonizing the lamina papyracea laterally and skull base (fovea ethmoidalis) superiorly under continuous optical magnification. The sphenoid sinus ostium was identified medial to the superior turbinate and enlarged. The frontal recess was cleared of obstructing polyps to establish wide patency.

Left Nasal Cavity: Mirror-image pathology was encountered. Uncinectomy, wide middle meatal antrostomy, complete ethmoidectomy, sphenoidotomy, and frontal recess clearance were completed in an identical fashion. Hemostasis was achieved with suction cautery and topical warm saline irrigations. Absorbable steroid-eluting sinus implants (Propel) were placed into both ethmoid cavities. Bilateral nasal patency was confirmed without structural damage to the lamina papyracea, orbit, or skull base. Blood loss was 60 mL.